

Experiences of children impacted by the 2019/2020 bushfires in South Australia: A qualitative study

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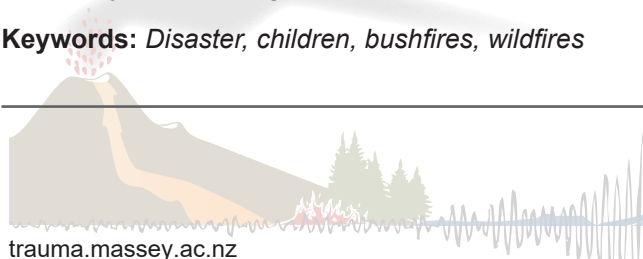
Abstract

Little is known about children's subjective experiences of bushfires. This study explores child experiences of South Australian bushfires that occurred during 2019/2020. Semi-structured interviews explored the experiences of children who lived through the fires in the Adelaide Hills (n = 7) and Kangaroo Island (n = 2) and were subsequently referred to a tertiary mental health service. Responses were thematically analysed from a psychological perspective. Five themes were generated: (1) planning, (2) separation, (3) fear of fire's destruction, (4) sense of a close call, and (5) the fire left its mark. Children reported similar bushfire experiences, and recalled the event as chaotic and frightening. Many experienced brief separations from caregivers without means to communicate and conveyed a deep concern for others' safety including family and pets. Clinical care practitioners should be aware of the specific fears and worries children experience during disasters, and particularly, whether children experienced separation from a caregiver. Furthermore, the presence and effect of pre-existing, co-occurring negative life events and secondary stresses ought to be considered.

Keywords: Disaster, children, bushfires, wildfires

In 2019, prolonged drought, above-average temperatures, and a series of dry lightning storms eventuated in a string of devastating climate-induced bushfires in Australia. Between November 2019 and February 2020, South Australia was severely impacted with almost 280,000 hectares burnt, 211,500 of which were on Kangaroo Island (Government of South Australia, 2020). In South Australia, the bushfires, commonly referred to as wildfires internationally, resulted in the deaths of three people, the destruction of 196 homes, the deaths of nearly 68,000 livestock, and substantial impacts on native habitats and wildlife loss (e.g., 50,000 koalas on Kangaroo Island; Government of South Australia, 2020). Furthermore, across South Australia, approximately 8,000 people were displaced throughout the bushfire event (Du Parc & Yasukawa, 2020). Whilst exact duration of displacement is unknown, it would be expected to range between a few hours to days and beyond especially given that bushfire recovery efforts have shown that house rebuilding can take between 1- 4 years (Australian Red Cross, 2020).

Children, young people, and adults may have similar disaster experiences, such as disruption and displacement, perceived threat, and harm or loss of life and belongings. Several methodological approaches have been used to understand and represent children's experiences and their post-disaster needs. These approaches include comparative observations by carers of infants residing in London in war nurseries with those living in typical circumstances (Freud & Burlingham, 1944), and adult (e.g., parent/teacher) completed questionnaires about disaster-related mental health impacts on children (see Spencer & Thompson's, 2024 scoping review). Children's participation in disaster research includes their completion of self-report mental health screeners and standardised questionnaires (e.g., Geng et al., 2019; Yelland et al., 2010). Furthermore, activities such as focus groups, walking tours, and photo-elicitation techniques have been used to garner children's views about their experiences and resilience building skills: for example, their knowledge and contributory roles to disaster responses and policies (e.g., Gibbs, Waters, et al., 2013; Walker, 2016). The CUIDAR (Cultures of Disaster Resilience among children



and young people) European Union-funded projects are examples of such endeavours (Mort et al., 2020).

Children may be disproportionately affected by their disaster experiences such as changes in their usual supports and social networks and ongoing disruptions that affect their safety and stability (Danese et al., 2020; Gibbs et al., 2015; Norris et al. 2002; Raphael & Burns, 2014). Short- and long-term effects include higher risk than adults of developing post-traumatic stress disorder (PTSD; Le Roux & Cobham, 2022; McDermott & Palmer, 2002; To et al., 2021; Yelland et al., 2010), depression (Brown et al., 2019), and longer-term psychological morbidity (McFarlane et al., 1987; McFarlane & Van Hoof, 2009). Given that surge demand for psychosocial services can outstrip organisational capacity, a stepped-care model that aligns needs with treatment intensity (i.e., universal, targeted, and specialist interventions) has been recommended (McDermott & Cobham, 2014; Phoenix Australia, 2020).

Provided through the Women's and Children's Health Network in South Australia, the Child and Adolescent Mental Health Service (CAMHS) is a state-wide tertiary mental health service which provides care to young people with severe and/or complex mental health needs. In response to the bushfires, CAMHS received funding from the Government of South Australia, via the Office of the Chief Psychiatrist, to create a multi-disciplinary Bushfire Response Team which provided services to bushfire-affected young people. Given the effect of other negative life events and co-morbidities on post-disaster trajectories and recovery (La Greca et al., 2013; Lai et al., 2013), referrals were accepted from key community agencies, including schools, and from parents/carers of children continuing to experience distress related to the fires and/or the extensive media coverage. The current study aimed to explore children's subjective experiences of the 2019/20 South Australian bushfires by analysing data from questionnaires and clinical interviews undertaken at least 6 months post-bushfires by the CAMHS Bushfire Response Team.

Methods

Participants and Setting

A total of 140 families from fire-affected regions were serviced by the CAMHS Bushfire Response Team, of which 13 families consented to participate in a research study. Nine participants are included in the current study. Four participants were excluded as questionnaires were answered by caregivers, rather than the child.

Participants were aged 5 to 14 years ($M = 10.33$, $SD = 2.44$), seven participants resided in the Adelaide Hills, and two on Kangaroo Island. Four participants were girls and five were boys. Given location and population size and to preserve participants' anonymity, further identifying details are not provided.

Data Collection

This study was approved by the Women's and Children's Health Network's Research Ethics Committee (2020/HRE01641). Data collection took place between August 2020 and September 2022. The data used in the current study involved caregivers and children completing questionnaires (e.g., Child PTSD Symptom Scale for DSM-5 (CPSS-V); Foa et al., 2018). Children wrote their responses to the CPSS-V and were supported by the interviewer who wrote their dictated responses in some cases if preferred by the child. The other data was from a semi-structured interview conducted by the treating clinician with the child, in the presence of a caregiver. This was recorded by note taking by the interviewer who was in all cases the treating clinician. Additional questionnaires that yield quantitative data were administered but are not included here as they are beyond the scope of the current study (see Appendix).

The CPSS-V requires the child to nominate a "scary or upsetting thing" that bothers them most when they think about it and then rate frequency and severity of reactions. Children were asked to self-nominate bushfire and non-bushfire related experiences. The semi-structured interview explored child and family experiences during the bushfire. The interview was devised by Dr Anne Gannoni (third author) based on available literature and in consultation with CAMHS staff and external experts and took approximately 40 minutes to complete. Most interviews were conducted face-to-face with one completed via telehealth. Questions included details about evacuation, damage to belongings, worries and fears during and after the event, involvement during fires, knowledge of and comparisons with peers' experiences, and the experience of returning home (see Appendix for semi-structured interview content).

The clinical interview was to be administered on intake, with the CPSS-V to be administered on intake, every 3 months during service engagement, and on discharge. However, there were discrepancies amongst participants in questionnaire and interview completion. Seven of the nine participants completed the clinical interview. Regarding the missing data - a clinical interview was completed only with parents for one child, and an

interview was not conducted with the remaining child. Eight children completed the CPSS-V. Completion of measures can be found in Table 1.

Table 1

Participant Completion Status of the Clinical Interview and CPSS-V

Participant	Clinical interview	CPSS-V
1	Not completed	Completed three (intake, 3 months, and discharge)
2	Completed	Completed two (intake and discharge)
3	Completed	Completed one (intake)
4	Completed	Completed two (intake and discharge)
5	Completed	Completed one (intake)
6	Completed	Not completed
7	Completed	Completed two (intake and discharge)
8	Not completed	Completed one (intake)
9	Completed	Completed one (intake)

Data Analysis

Reflexive thematic analysis (TA; Braun & Clarke, 2022) was selected to explore child experiences of the fires. Given its bottom-up analytical approach, reflexive TA allows for an inductively-oriented analysis, focused on finding patterned semantic and latent meanings. The analysis was conducted by the first author. The process involved familiarisation with and thematical analysis of the data, which included the child's self-nominated bothersome bushfire and non-bushfire experiences (CPSS-V; Foa et al., 2018) and responses to the semi-structured interview. Using NVivo12 software for coding, the data set was worked through systematically four times, each time in a different order to avoid an unevenly coded data set (Braun & Clarke, 2022). Initial candidate themes were generated, reviewed, and refined. The final themes that were derived were based upon their relevance to the research aim and discussed with two supervising researchers with background knowledge in the research project and thematic analyses.

Results

Five themes were generated through the analysis process: (1) planning, (2) separation, (3) fear of fire's destruction, (4) sense of a close call, and (5) the fire left its mark (Refer to Table 2 for themes and sub-themes developed through thematic analysis).

Table 2

Themes and Subthemes Developed Through Thematic Analysis

Theme	Subthemes
Planning	Evacuation plans as reactive Evacuation plans as proactive
Separation	Communication during evacuation
Fear of fire's destruction	Fear of losing belongings Fear of losing the house Fear for others' safety
Sense of a close call	
Fire left its mark	Lasting environmental impact Lasting psychological impact

Planning

Children were asked about where they were when they heard about the fires and what subsequent actions were taken. Children provided details about whether they had an evacuation plan and if they adhered to that plan. One participant described not feeling worried during the evacuation as they followed their family's bushfire plan:

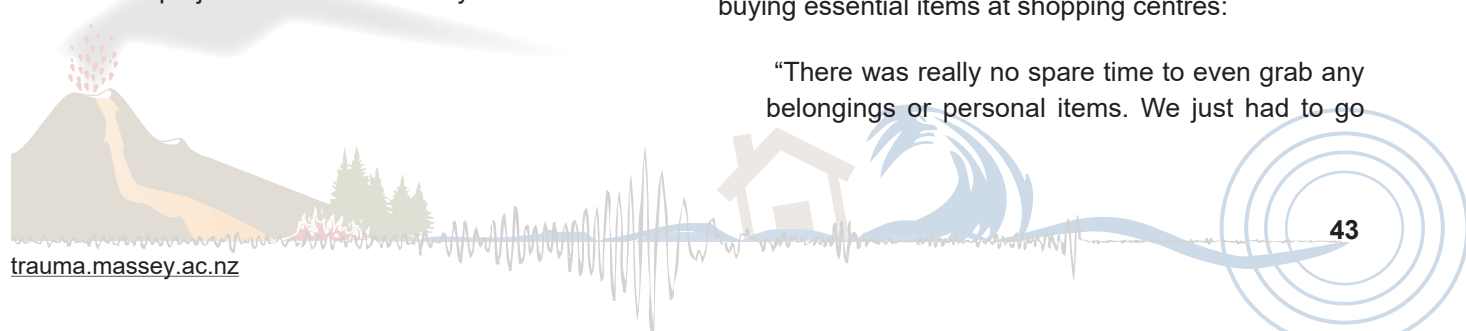
"Not worried. I knew we were at Grandma's and fires didn't go that way and we were safe there and that's where we go if we need to evacuate."

Overwhelmingly, however, accounts from the children describe that there was not enough time to enact fire plans and commonly experienced a rushed and reactive evacuation. For example, an interviewer noted the following about a child's experience:

"Family had planned to leave, though left it too late to leave and took refuge on the sports oval where people were using wire cutters to open the gates".

Children reported that original plans had to be modified due to the rapidly evolving threat. This is implied by the children's accounts of having to leave their homes quickly, often without belongings, and subsequently buying essential items at shopping centres:

"There was really no spare time to even grab any belongings or personal items. We just had to go



quickly.” Additionally, “[we] had to go to [shopping centre] for toothbrushes and phone chargers as we couldn’t go home to get belongings”.

These experiences create a sense that the fire danger evolved faster than anticipated or that evacuation was unexpected. This is again reinforced by children recounting that they were away from their homes when fire warnings were issued, and thus unable to follow original bushfire plans:

“Family has plan for when at home, but no plan for if out and have to evacuate”.

Additionally: “[I was] watching Star Wars at cinema. Mum got all these messages when we got out. Went go grandma’s [...] no belongings.”

These accounts demonstrate that children were aware of their family’s bushfire plans, and aware if they were not followed. Children conveyed that many of their families were caught off guard by the fire. This is further supported by most children recalling their parents had left for work on the morning of the fires. This led to most children being separated from family during the evacuation process, with separation discussed next as another main theme.

Separation

During evacuation, most children experienced separation from a family member at some point. Across participant age groups, periods of separation were commonly described as eliciting “worried” and “scared” emotions, with children often not knowing where their family were and whether they were safe. For example, one child stated:

“Can’t remember if Dad was at work. Worried about where he was. Better when [Dad] returned because everyone there to hug me because I was crying and sad.”

While they were separated, children described attempting to keep in contact with family members, with communication emerging as a subtheme. Participants described having to listen to the “scanner”¹ and “radio”, as they had “no power”, “no internet”, and “no service”. This made communicating with separated family members difficult and sometimes impossible during evacuation. One child stated:

“Mum at home till left for work [...] lost communication with mum on freeway”.

Once evacuated, many of the children’s parents left to help in different capacities, such as assisting neighbours, fighting the fires, or collecting supplies. Children conveyed immense fear for their parents’ safety, for example:

“I left and mum tried to put fire out. I was scared I would never see my mum again”.

Overall, separation occurred for varying reasons and at different points of the evacuation process. However, regardless of these differences, the experience of separation was commonly described as causing children distress and eliciting concern for the safety of the people from whom they were separated. Indeed, many children spoke about fearing for others’ safety during the fires, demonstrating a keen awareness of the fire’s destructive potential. Fear of the fire’s destruction was subsequently generated as a main theme.

Fear of the Fire’s Destruction

Children commonly described fear of the fire’s destructive capabilities, including the risk it imposed on the safety of their family members. During the semi-structured interview, children were asked if they were worried about their family members being hurt. All of them indicated that they were and listed the people they were worried about. Children listed their mum, dad, siblings, grandparents, and pets. Children responded that they were: “worried about dad”, “scared I would never see my mum again”, and “mum getting stuck” and “dad getting more hurt”. In addition, four of the seven children who completed interviews (which took place from 10 months to 2 years post disaster), rated their worry about others’ welfare at the time of the fires in the very high range (i.e., 8-10, with 10 labelled as “Heaps worried”). Also taken at interview, their ratings reflecting their current worry about others’ welfare were placed in the mid-range (i.e., 3-7), suggesting ongoing concern despite the time-lapse. In contrast, when asked if they were worried for themselves at the time (e.g., getting hurt), several children responded “no” or did not respond, with one participant explaining that their “mum and dad had it under control”. On the other hand, two children indicated that they were very worried for their personal safety (i.e., ratings 8-10, with 10 labelled as “Heaps worried”). Children’s descriptions of the psychological effects of their experiences are included in the theme, “The Fire left its Mark”.

Multiple children described concerns for their pets they had to leave behind. One child wrote she feared her pet would “wander into the fire”. Another consistently

¹ A scanner is a radio receiver that can automatically tune, or scan two or more frequencies permitting connection to services, (e.g., Country Fire Service) and businesses.

referenced his concerns for the family dog: “worried and scared for Buster²” and still “I worry about Buster a lot, that he’s going to die”. Another child conveyed his concern for the local koalas, describing how he cared for them after the fires:

“[I] helped save koalas in my cart [...] I saved lots of them, gave water, but they were very sick. Some went to the wildlife park.”

These accounts indicate that the children had an acute awareness of family members and animals being in a life-threatening situation. In addition, most children conveyed fears of losing their homes, with children commonly saying that “the house burning down” was one of the scariest thoughts they had during the fires.

In addition to their homes, children referenced fear of losing their personal belongings. For example, one child described the fires as:

“Really scary, worried about my stuff, didn’t want it to burn down”.

It is evident the children knew of the fires’ potential to harm people, animals, and their homes, and this was a common fear shared amongst participants. The majority of participants’ homes were unscathed by the fire, but many commented on the proximity of the fire to their properties, conveying they felt it was a “close call” and a worse outcome had been anticipated. This was subsequently identified as a main theme.

Sense of a Close Call

A key theme generated was a shared sense amongst most participants that they narrowly escaped the fire’s destruction. This theme derived from children’s descriptions of their feelings of relief and gratitude when returning home implies they had expected or imagined losing their houses and belongings. For example, when asked about their return home, one child said:

“Relieved to see house to believe it wasn’t gone [...] Almost did, fire came to grass”.

Similarly, an interviewer noted the following about another child’s experience:

“Very emotional, saw devastation, very upset. Filmed roadside on the way home. Grateful to still have a house.”

One child gave details of the fire’s proximity to their home and compared their experience to that of their neighbours:

“House opposite- gardens and veranda burnt. House diagonally- opposite burnt, 200 meters away- and all of the trees. If the CFS (Country Fire Service) were not there, would have lost house.”

This account demonstrates a child contextualising their experience in relation to their neighbours and using it as an example of what they themselves narrowly avoided. Additionally, many children described evidence of the fire being near their homes, such as ash in their backyards and swimming pools, bins melted on the side of the road, and views of “blackness” from their homes. These descriptions, coupled with reported emotions of relief and gratitude, imply many children felt it was a close call regarding the survival of their homes and belongings as well as their parents or pets.

The Fire Left its Mark

After the immediate threat of the bushfires had passed and families returned home, children described how the fires left a lasting mark, both environmentally and psychologically. Many participants commented on the changed landscape near their homes with one interviewer noting the following about a child’s description of the fire damage:

“Regrowing but can still tell where the fires were, trees still more sparse”.

Indeed, many children described being surrounded by visual reminders of the bushfire’s impact, for example, explaining their area lost “80% of trees”, or the environment is now “black” and “burnt” with “nothing around”.

In addition to the physical marks left by the fires, children described persisting psychological impacts. One child whose house “totally burnt down” despite parental efforts to save it, expressed that since the fire, they become worried when separated from their mother:

“I worry when I’m not with my mum. I worry that if I feel sick or have a tummy ache or feel nausea coming on I can’t trust anyone else to care for me.”

Two children described recurring nightmares about the fires: “I get nightmares about the fires, and I can hear the birds crying from burning.” Another child, who had been co-sleeping with parents since the bushfires, stated: “[I]

2 Name changed for confidentiality.

Dream fire coming over the back, need to get out [as] fire gets closer.”

Naturally, the fires were a topic of discussion at school, for example, one child stated:

“Back at school - people talking about it A LOT. Talking about what they did”.

Further to the child’s account about koala rescues, two other children described how they helped others, one through volunteering with family at the donation centre sorting clothes and the other:

“[Sibling] and me have a few friends affected by the fire so we took extra handouts, and gave to our friends”.

Most children said they knew of community members who had suffered burns or other physical and mental health problems or had died since the fires through non-bushfire related events. To ascertain the effect of the co-occurring COVID-19 pandemic on children, a 2-item questionnaire to be administered at the interview had been included. Children’s ratings about the extent of their worry in the past month were placed in the lower range [i.e., 0 = not at all to 5 = (somewhat) worried] with the “things [they] most worried about” being “getting sick”, “not seeing family”, “not seeing friends”, and “not being able to do my usual things”. Moreover, some children’s self-nominated current bothersome non-bushfire related worries/experiences included: continued bullying at school for over 12 months, the unexpected death of a family friend, and thoughts of significant harm to and death of parents and self. Notably, some children in our study reported symptoms congruent with PTSD criteria such as nightmares. This suggests the possibility of persistent and potentially exaggerated negative beliefs about the world being more dangerous. Overall, children’s accounts demonstrate the bushfire’s persisting physical reminders and psychological effects, with co-occurring stressors and negative events (including the COVID-19 pandemic), maintaining hypervigilance, and a sense of threat, harm, and mortality to self and others.

Discussion

This study aimed to explore the experiences of South Australian children who were impacted by the 2019/2020 bushfires and referred to a tertiary mental health service. Previous research has predominantly focused on the development and presence of specific mental health disorders in young people, with their

subjective experiences often unexplored or reported upon (Self-Brown et al., 2017). Yet recognising children’s perspectives, especially the meaning and the effects of disasters, may promote their sense of value and belonging to community and importantly inform organisational service delivery to meet their needs.

This study extends existing knowledge through analysis of children’s accounts of their experiences during the fires and uniquely represents child voices from a tertiary mental health service context. Children indicated that overall, their experiences of the bushfires were chaotic and frightening, whether it was while families prepared to leave, subsequent evacuation, separation, or upon their return home. Experiences of separation were reported as common and distressing. Additionally, concern and awareness of “others” was a salient thread that wove together aspects of the children’s reported experiences. “Others” include parents, siblings, pets, local animals, and by extension, the wider living environment. The reactions of some children were further amplified by their non-bushfire experiences which included pre-existing and co-occurring secondary stressors such as the COVID-19 pandemic.

Loss of Communication During Evacuation

Brief separation was found to be a common and stressful occurrence, with children highlighting struggles to stay in contact with family members due to damaged infrastructure. Richardson et al. (2016) posited that society’s collective reliance on mobile devices may amplify the feelings of disconnection and uncertainty when they falter during such events, particularly for children who are sometimes referred to as “digital natives” (Bittman et al., 2011). Indeed, uncertainty is known as a contributing factor to anxiety (Gu et al., 2020) and, therefore, it stands to reason that reducing aspects of uncertainty may reduce some distress experienced by children in these circumstances. For example, several children mentioned their parents using other forms of technology to stay informed of the developing situation like the radio or the “scanner”. Given children’s status as “digital natives”, they may be well positioned to learn and instruct others about alternative technologies. Indeed, in their study investigating children’s experience of a bushfire education program, Gibbs et al. (2018) quoted one child who had since taught their mother to use radios, demonstrating their ability to give appropriate advice to parents. Although uncertainty and separation appear inevitable in the context of bushfires, children within our study point to ways this may be reduced. Previous studies also offer support for involving children

in these processes (e.g., Pfefferbaum et al., 2018, gives examples of children's participation in activities across preparedness, response, and recovery stages).

Brief separation

Although established as common and stressful, research about experiencing separation during disasters has largely explored the impact of longer-term separation caused by displacement (Hansel et al., 2013; Lau et al., 2010; McFarlane et al., 1987; Richardson et al., 2016; Usami et al., 2012). Yet, due to the unpredictable and rapidly developing nature of the bushfire, seeking safety from attachment figures was at times thwarted for the children in this study. Additionally, they shared stories of separation and concern for their pets, which are also regarded as attachment figures (Groenewoud et al., 2023). However, despite reunification, children reported ongoing anxieties about their pets and family. Thus, the potential of brief separation to lead to changes in attachment style is important to consider, as such disruptions have been linked to poorer behavioural, academic, and mental health outcomes (Schröder et al., 2019). As such, Bryant (2023) has recommended that models of PTSD incorporate attachment theory and social processes into biological, learning, and cognitive processes, given their important contribution to PTSD aetiology, maintenance, and outcome.

Social Comparison

Aside from their own bushfire experiences, children provided examples of the experiences of "others" (e.g., neighbours, school mates, or local animals), some of whom had been injured, lost their homes, or passed away. Drawing upon others' experiences and measuring it against their own may demonstrate a process of social comparison (Festinger, 1954). Downward comparisons made with others who are in the same or less fortunate situations generally occur when there is a threat to self-esteem and can serve self-protective and enhancement purposes (Wills, 1981). For adults within a post-disaster context, the role of social comparison as a cognitive coping strategy contributed, as highlighted by Brady et al. (2021), to the establishment of explicit and implicit hierarchies of affectedness. In turn, these had the potential to influence personal and community consequences such as contrasting validations of experience, external agencies' responses, and the availability of resources including entitlements (Brady et al., 2021). These authors also noted that for some survivors, these processes served as a helpful sense-making tool, whereas for others, it would be a source of

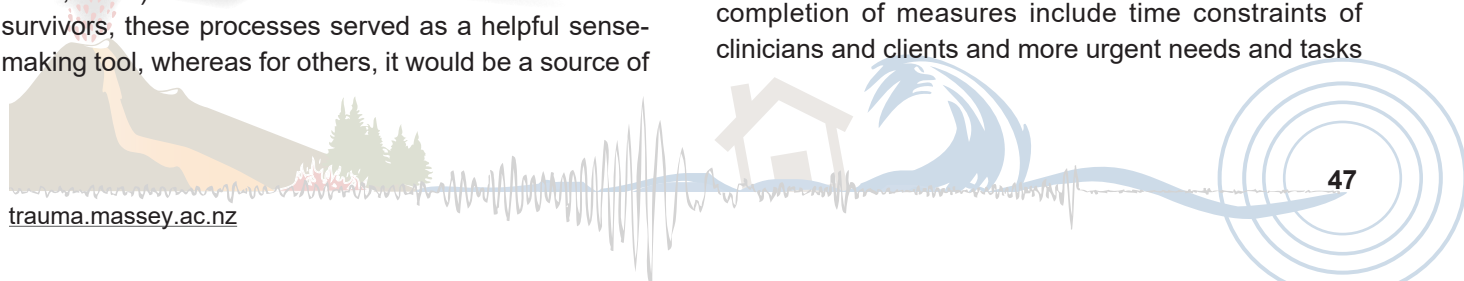
secondary stress whereby they felt judged regarding how badly they were affected compared to others. In our study, children often referenced others who they deemed were "worse off" than themselves, demonstrating downwards social comparison. In contrast (i.e., reflective of upward social comparison), for one child who lost their house, a sense of anger and injustice is implied in their description of their affectedness relative to less impacted friends ("my house burnt down where my friends only had (to) evacuate"). Like other research (e.g., Mort et al., 2018) our theme, "sense of a close call", may have captured part of this social comparison process, whereby for most children, their comparative perspectives demonstrate feelings of relief for their position post-disaster, alongside more complex realisations relating to threats, mortality, and safety.

Changes to the Environment

Coinciding with their reports of others' experiences, children often referenced changes to their home environment and places around them, including damage to neighbouring properties and their surroundings, using descriptors like "burnt" and "black". Their subjective accounts demonstrate that they indeed care for their local environments and animals, and damage or injury to them elicits distress. This may suggest vulnerability to experiencing solastalgia, a concept describing distress due to the perception and/or degradation of inhabited environments and the loss of comfort the environment once provided (Block et al., 2019; Hinds & Sparks, 2008; Leviston et al., 2023). Whilst not evident in this study, we acknowledge the broader scope of place connections (e.g., Scannell et al., 2016; Summers et al., 2019) and which for First Nations peoples relates inherently to spiritual and cultural attachments and practices (Breth-Petersen et al., 2023; Williamson et al., 2020).

Limitations and Strengths

Within the current study, several limitations that reflect challenges faced when conducting research in post-disaster settings were identified. These include a low participation rate and the method and completion of measures. Irrespective of methodological approaches (i.e., quantitative and qualitative), low participant numbers and recruitment challenges have been reported in many studies (e.g., Cobham & McDermott, 2025; Delicado et al., 2017; Spencer & Thompson, 2024). Several factors that likely affected the method and completion of measures include time constraints of clinicians and clients and more urgent needs and tasks



taking precedence for families during disaster recovery periods (Knack et al., 2006). However, despite these challenges, the current study attempted to make best use of responses available, ensuring the experiences were honoured as rich and informative sources of data. This can be taken as a relative strength of the study, whereby despite challenges posed by research in a post-disaster context, child voices were placed at the forefront.

Additionally, participants within the current study were receiving care from a tertiary mental health service and may therefore be representative of children who were more psychologically affected by the fires. As the intention of the study was to explore experiences within this cohort, it is an important point to consider when comparing current findings with other study populations. Including children's self-nominated non-bushfire worries and child-perceived impact of the COVID-19 pandemic further strengthened the study as previous research has shown that the experiences for adults (Cowlshaw et al., 2024) and adolescents (Beames et al., 2023) of bushfires and pandemics as compound disasters are disruptive of recovery processes.

Implications and Conclusion

This study heeds recommendations to adopt a child-centred approach made by researchers (Gibbs, MacDougall et al., 2013; Harms et al., 2021; Peek, 2008), government bodies (Australian Government National Emergency Management Agency, 2020), and non-government organisations (Royal Far West and UNICEF Australia, 2021). The study captures the complex cumulative experiences of some children who lived through the 2019/2020 South Australian bushfires and were referred to a tertiary mental health service. Children described separation and disrupted communication to be particularly distressing. This highlights potential practical and clinical implications: a need to mitigate and plan for separation and communication, how separation may be experienced by the child as a distressing psychological event, and what can be done retrospectively to repair this.

Turning now then to specialist level interventions, the Trauma-Focused Cognitive Behavioural Therapies (e.g., CBT and Eye Movement Desensitisation and Reprocessing) have the strongest evidence base (McDermott & Cobham, 2014; Phoenix Australia, 2020), but with limited interventions and evidence available for younger children and those with disabilities (Gibbs et al., 2021). However, Bryant's (2023) recommendation

to include attachment theory and social processes into PTSD models underscores the need to provide relational/attachment-based therapies. An example and included in the CAMHS Bushfire Response Team's intervention framework is Parallel Parent Child Narrative (PPCN), stage 1 of the 4-stage process of Parallel Parent and Child Therapy (P-PACT) developed by Heather Chambers. Still requiring formal evaluation, PPCN's key approach is working with parent/child dyads to undertake a form of therapeutic storytelling. The focus is on the relationship, on revealing mistakes of meaning, countering hurt, shame, and blame, and proving good intentions for parent and child. It has been used in New Zealand and Australia since the early 2000s across private practice and governmental services (including CAMHS) and found to be an acceptable and beneficial intervention by families and therapists (Amos, 2024; Furber et al., 2013). Furthermore, given that children engaged in social comparison after the fires, relational/attachment therapies may provide opportunities to support conversations about community hierarchies of affectedness.

The data analysed for the current study was drawn from a data set which included parent reports of the bushfire. Continuing with the theme of relational understandings, we recommend a comparison between parent and child reports as these may provide useful insights into how events are perceived from differing perspectives.

Considering children are particularly vulnerable to the threats posed by climate-induced disasters and the knowledge that these events are predicted to increase (United Nations Environmental Programme, 2022), it is pertinent to continue including children in research, especially when they are service recipients, and to look for ways to include them as co-researchers (Gibbs, Mutch, et al., 2013).

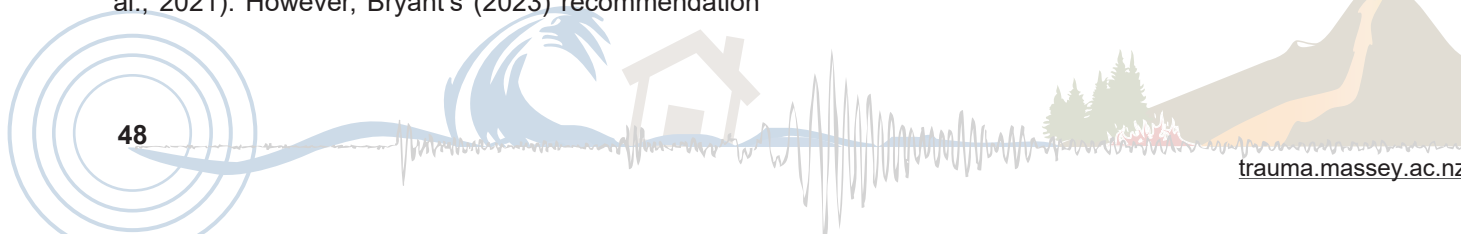
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Authorship Statement

Stella Lycos: Conceptualisation, data curation, formal analysis, methodology, visualisation, writing- original draft, writing- review and editing.

Rachel Roberts: Conceptualisation, project administration, resources, supervision, writing-review and editing.

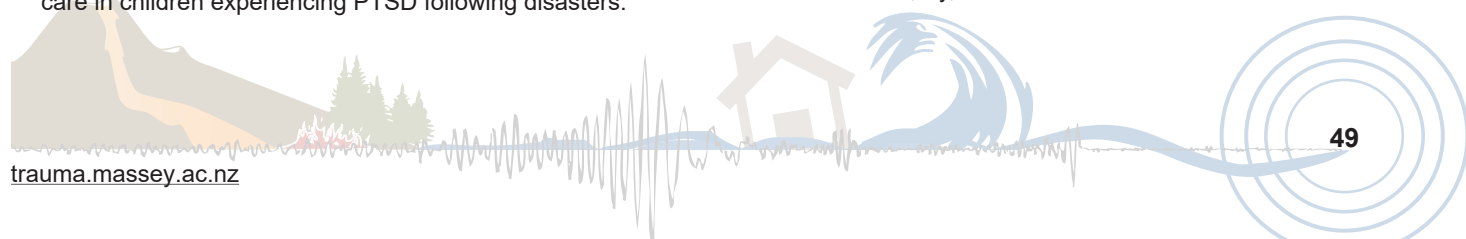


Anne Gannoni: Conceptualisation, data curation, investigation, project administration, resources, supervision, writing-review and editing.

Susan Hamilton: Data curation, writing- review and editing.

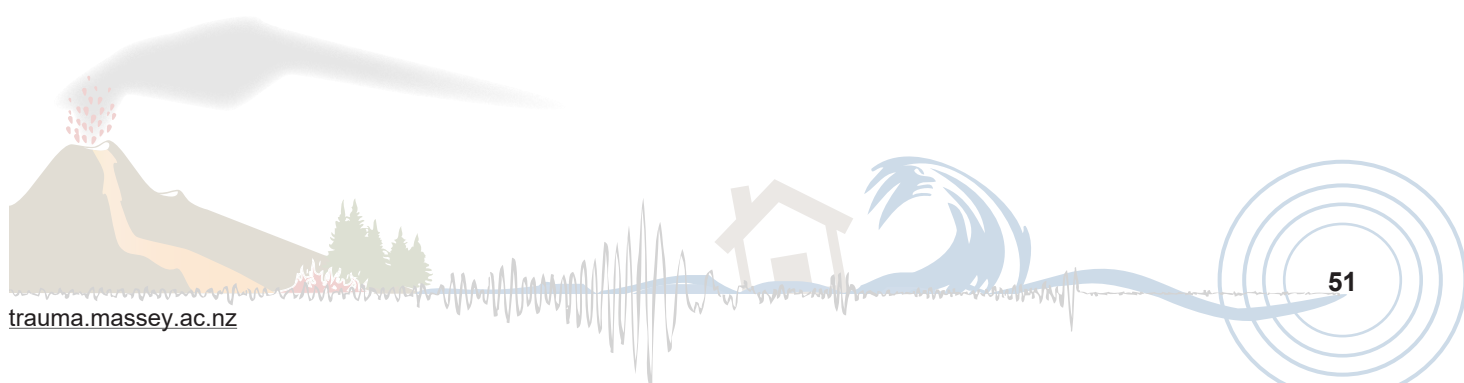
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Appendix

Protocol Questionnaires

- Initial consultation and allocated clinician appointments

Revised Child Impact of Events Scale (CRIES-8)

The Children's Revised Impact of Events Scale (CRIES-8) is a brief child-friendly measure designed to screen children at risk for posttraumatic stress disorder (PTSD; Perrin et al., 2005). It is a worldwide measure used following large and small-scale disasters, has good reliability, a stable factor structure and has been administered to children as young as 5 years old.

Child PTSD Symptom Scale (CPSS-V): Child and Parent Versions

The CPSS-V is a 20 item self-report measure of child PTSD for young people aged 8-18 years of age (Foa et al., 2018). A further seven items focus on daily function, (e.g., relationships with friends, schoolwork) to yield a total impairment score. The CPSS-V is reported to have excellent internal consistency and good test-retest reliability.

PTSD Checklist for DSM-5 (PCL-5)

The PCL-5 is a 20-item parent self-report measure that assesses the presence and severity of PTSD symptoms in adults in the past month (Weathers et al., 2013). Items correspond with DSM-5 criteria for PTSD and the measure takes about 5-10 minutes to complete and has sound psychometric properties.

Additional questions: Semi-structured interview

Questions to garner children's experiences of the fires were devised for the study. Although written in a semi-structured format, the questions are asked in a child-friendly conversational style and parent/caregiver could be present.

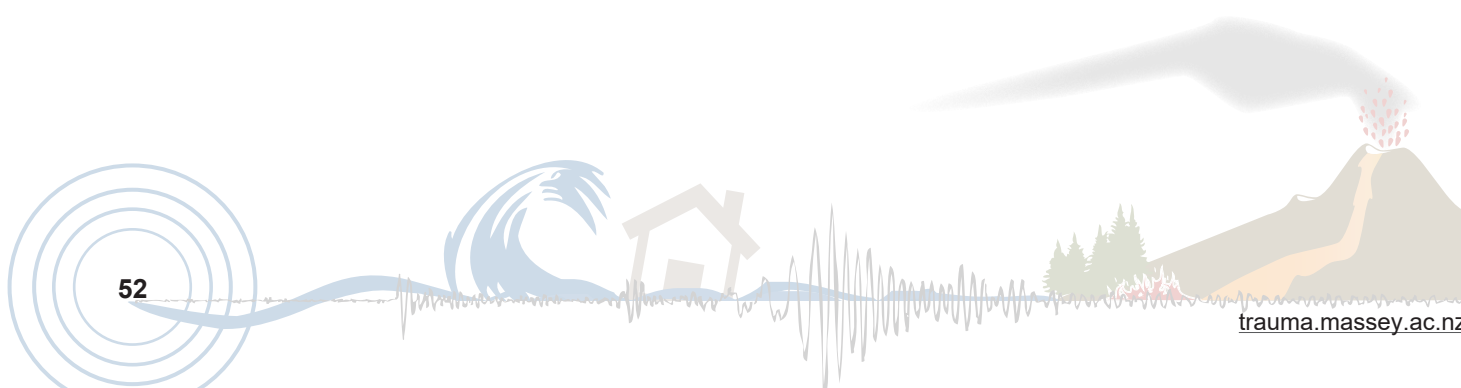
Two brief questionnaires were devised to be administered early in the service contact:

- 1) Covid 19, i.e., a worry rating and the things most worried about and
 - 2) Pre-Bushfire functioning: i.e. use of, and recommended need for psychosocial services pre-bushfire.
- Repeat administration at 3 months until discharge related to service delivery requirements.

CPSS-V (Child and Parent version), PCL-5, and semi-structured interview (reduced items depending on treatment phase and status).

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Additional questions

These questions cover several areas: evacuation, damage to belongings, worries and fears during and after the event, involvement during fires, knowledge of, and comparisons with peers' experiences, and the experience of returning home. The wording and sequence of questions and topics may be altered for relevancy and to promote child friendly conversation, and normal flow. (Some examples of follow up topics/questions are provided in brackets).

"I would like to talk with you about what happened for you and your family when the bushfires happened at Christmas time."

- Were you and your family at home when the bushfires happened or were you somewhere else?
- Using a map, identify location of home and proximity to fires. Record any other details.
- Did you stay at home, or leave when the bushfires were happening?
- At home, were you trapped and unable to leave, separated from parents/caregivers?

(What was that like for you, other members of the family? (e.g., who was at home, what did you/they do; If separated, who were you with, duration, hearing about family members whereabouts, welfare; what were your experiences/ reactions at the time, other family members reactions).

- If left, was that planned or unplanned? Please tell me what happened. (e.g., what did you/family members have to do?)
- If evacuated, location, duration of evacuation, with whom?
- Was any of your property damaged? (Describe damage/ loss and ask about child's belongings).
- Do you have any animals, pets/stock? what happened to the animals? What did you see?
- If damage and/or stock loss, what is happening now with your property/stock/land?
- Was any member of your family involved in helping with the fires? Were you involved?
- Did anyone you know get hurt in the bushfires? Tell me what happened.
- Were you worried that other members of your family would get hurt? Who? How worried were you about them? How worried are you now about them?
- Were you worried that you would get hurt? How worried were you? 0 (not at all) -10 heaps worried)
- Did you have the same or different experiences to your friends and classmates? In what way was it the same/different?

Follow up questions

- To what extent has life returned to normal for you? Rate 0 (not at all)-10 (back to normal)?
- Please tell me what has and hasn't got back to normal for you, your family.

